

KLONOWSKI,JAMES

DOB: 06/07/1988
 Sex: M
 Phone: (630) 864-0965
 Patient ID: 89865297

Age: 37
 Fasting: Y

Specimen: ZW785972V
 Requisition: 0004342
 Lab Reference ID: 3399781
 Report Status: FINAL / SEE REPORT

Collected: 05/30/2026 11:30
 Received: 05/30/2026 11:30
 Reported: 06/03/2026 10:27

Client #: 73943516
 SOERRIES,SCOTT
 WHOOP-CIT
 10101 RENNER BLVD
 LENEXA, KS 66219-9752
 Phone: (844) 738-3787

FASTING: YES

▲ IRON, TIBC AND FERRITIN PANEL

Analyte	Value	
IRON, TOTAL	62	Reference Range: 50-180 mcg/dL
IRON BINDING CAPACITY	317	Reference Range: 250-425 mcg/dL (calc)
% SATURATION	20	Reference Range: 20-48 % (calc)
▲ FERRITIN	36 L	Reference Range: 38-380 ng/mL

▲ COMPREHENSIVE METABOLIC PANEL

Analyte	Value	
GLUCOSE	86	Reference Range: 65-99 mg/dL
Fasting reference interval		
UREA NITROGEN (BUN)	14	Reference Range: 7-25 mg/dL
▲ CREATININE	1.31 H	Reference Range: 0.60-1.26 mg/dL
EGFR	72	Reference Range: > OR = 60 mL/min/1.73m2
BUN/CREATININE RATIO	11	Reference Range: 6-22 (calc)
SODIUM	139	Reference Range: 135-146 mmol/L
POTASSIUM	4.3	Reference Range: 3.5-5.3 mmol/L
CHLORIDE	104	Reference Range: 98-110 mmol/L
CARBON DIOXIDE	29	Reference Range: 20-32 mmol/L
CALCIUM	9.4	Reference Range: 8.6-10.3 mg/dL
PROTEIN, TOTAL	6.8	Reference Range: 6.1-8.1 g/dL
ALBUMIN	4.3	Reference Range: 3.6-5.1 g/dL
GLOBULIN	2.5	Reference Range: 1.9-3.7 g/dL (calc)
ALBUMIN/GLOBULIN RATIO	1.7	Reference Range: 1.0-2.5 (calc)
BILIRUBIN, TOTAL	0.5	Reference Range: 0.2-1.2 mg/dL
ALKALINE PHOSPHATASE	60	Reference Range: 36-130 U/L
AST	14	Reference Range: 10-40 U/L
ALT	15	Reference Range: 9-46 U/L

VITAMIN D,25-OH,TOTAL,IA

Analyte	Value
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VITAMIN D,25-OH,TOTAL,IA**57** Reference Range: 30-100 ng/mL

Vitamin D Status 25-OH Vitamin D:

Deficiency: <20 ng/mL
Insufficiency: 20 - 29 ng/mL
Optimal: > or = 30 ng/mL

For 25-OH Vitamin D testing on patients on D2-supplementation and patients for whom quantitation of D2 and D3 fractions is required, the QuestAssured(TM) 25-OH VIT D, (D2,D3), LC/MS/MS is recommended: order code 92888 (patients >2yrs).

COMMENT

See Note 1

Note 1

For additional information, please refer to <http://education.QuestDiagnostics.com/faq/FAQ199> (This link is being provided for informational/ educational purposes only.)

TESTOSTERONE, FREE (DIALYSIS), TOTAL (MS) AND SEX HORMONE BINDING GLOBULIN**Analyte****Value****TESTOSTERONE, TOTAL, MS****429** Reference Range: 250-1100 ng/dL

For additional information, please refer to <https://education.questdiagnostics.com/faq/FAQ165>
(This link is being provided for informational/educational purposes only.)
(Note)

This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by the FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.

TESTOSTERONE, FREE**59.8** Reference Range: 35.0-155.0 pg/mL

(Note)
This test was developed and its analytical performance characteristics have been determined by Quest Diagnostics. It has not been cleared or approved by the FDA. This assay has been validated pursuant to the CLIA regulations and is used for clinical purposes.

MDF
med fusion
2501 South State Highway 121,Suite 1100
Lewisville TX 75067
972-966-7300
Ithiel James L. Frame, MD, PhD

SEX HORMONE BINDING GLOBULIN**36** Reference Range: 10-50 nmol/L**LIPID PANEL, STANDARD****Analyte****Value****CHOLESTEROL, TOTAL****111** Reference Range: <200 mg/dL**HDL CHOLESTEROL****51** Reference Range: > OR = 40 mg/dL**TRIGLYCERIDES****69** Reference Range: <150 mg/dL**LDL-CHOLESTEROL****45** mg/dL (calc)

Reference range: <100

Desirable range <100 mg/dL for primary prevention;
<70 mg/dL for patients with CHD or diabetic patients
with > or = 2 CHD risk factors.

LDL-C is now calculated using the Martin-Hopkins calculation, which is a validated novel method providing better accuracy than the Friedewald equation in the estimation of LDL-C.
Martin SS et al. JAMA. 2013;310(19): 2061-2068
(<http://education.QuestDiagnostics.com/faq/FAQ164>)

CHOL/HDL-C RATIO	2.2	Reference Range: <5.0 (calc)
NON HDL CHOLESTEROL For patients with diabetes plus 1 major ASCVD risk factor, treating to a non-HDL-C goal of <100 mg/dL (LDL-C of <70 mg/dL) is considered a therapeutic option.	60	Reference Range: <130 mg/dL (calc)

LIPOPROTEIN (a)

Analyte	Value
LIPOPROTEIN (a) Reference Range <75 Risk: Optimal <75 Moderate 75-125 High >125 Cardiovascular event risk category cut points (optimal, moderate, high) are based on Tsimika S. JACC 2017;69:692-711.	17 nmol/L

CBC (INCLUDES DIFF/PLT)

Analyte	Value
WHITE BLOOD CELL COUNT	7.8 Reference Range: 3.8-10.8 Thousand/uL
RED BLOOD CELL COUNT	4.89 Reference Range: 4.20-5.80 Million/uL
HEMOGLOBIN	14.6 Reference Range: 13.2-17.1 g/dL
HEMATOCRIT	45.4 Reference Range: 39.4-51.1 %
MCV	92.8 Reference Range: 81.4-101.7 fL
MCH	29.9 Reference Range: 27.0-33.0 pg
MCHC	32.2 Reference Range: 31.6-35.4 g/dL
RDW	12.9 Reference Range: 11.0-15.0 %
PLATELET COUNT	283 Reference Range: 140-400 Thousand/uL
MPV	9.1 Reference Range: 7.5-12.5 fL
ABSOLUTE NEUTROPHILS	4742 Reference Range: 1500-7800 cells/uL
ABSOLUTE LYMPHOCYTES	2168 Reference Range: 850-3900 cells/uL
ABSOLUTE MONOCYTES	569 Reference Range: 200-950 cells/uL
ABSOLUTE EOSINOPHILS	289 Reference Range: 15-500 cells/uL
ABSOLUTE BASOPHILS	31 Reference Range: 0-200 cells/uL
NEUTROPHILS	60.8 %
LYMPHOCYTES	27.8 %
MONOCYTES	7.3 %
EOSINOPHILS	3.7 %
BASOPHILS	0.4 %

HS CRP

Analyte	Value
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HS CRP

1.6 mg/L

Reference Range
Optimal <1.0
Jellinger PS et al. Endocr Pract.2017;23(Suppl 2):1-87.
For ages >17 Years:
hs-CRP mg/L Risk According to AHA/CDC Guidelines
<1.0 Lower relative cardiovascular risk.
1.0-3.0 Average relative cardiovascular risk.
3.1-10.0 Higher relative cardiovascular risk.
Consider retesting in 1 to 2 weeks to
exclude a benign transient elevation
in the baseline CRP value secondary
to infection or inflammation.
>10.0 Persistent elevation, upon retesting,
may be associated with infection and
inflammation.

Pearson TA, Mensah GA, Alexander RW, et al. Markers
of inflammation and cardiovascular disease:
application to clinical and public health practice:
A statement for healthcare professionals from the
Centers for Disease Control and Prevention and the
American Heart Association. Circulation 2003; 107(3):
499-511.

HOMOCYSTEINE

Analyte	Value
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HOMOCYSTEINE

7.5 Reference Range: < or = 13.5 umol/L

Homocysteine is increased by functional deficiency of
folate or vitamin B12. Testing for methylmalonic acid
differentiates between these deficiencies. Other causes
of increased homocysteine include renal failure, folate
antagonists such as methotrexate and phenytoin, and
exposure to nitrous oxide.
Selhub J, et al., Ann Intern Med. 1999;131(5):331-9.

CORTISOL, TOTAL

Analyte	Value
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CORTISOL, TOTAL

11.0 mcg/dL

Reference Range: For 8 a.m.(7-9 a.m.) Specimen: 4.0-22.0
Reference Range: For 4 p.m.(3-5 p.m.) Specimen: 3.0-17.0
* Please interpret above results accordingly *

DHEA SULFATE

Analyte	Value
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DHEA SULFATE

122 Reference Range: 93-415 mcg/dL

INSULIN

Analyte	Value
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INSULIN

3.2 uIU/mL

Reference Range < or = 18.4

Risk:
Optimal < or = 18.4
Moderate NA
High >18.4

Adult cardiovascular event risk category
cut points (optimal, moderate, high)
are based on Insulin Reference Interval
studies performed at Quest Diagnostics
in 2022.

TSH

Analyte	Value
TSH	1.67 Reference Range: 0.40-4.50 mIU/L

ESTRADIOL

Analyte	Value
ESTRADIOL	<30 Reference Range: < OR = 39 pg/mL

Reference range established on post-pubertal patient population. No pre-pubertal reference range established using this assay. For any patients for whom low Estradiol levels are anticipated (e.g. males, pre-pubertal children and hypogonadal/post-menopausal females), the Quest Diagnostics Nichols Institute Estradiol, Ultrasensitive, LCMSMS assay is recommended (order code 30289).

Please note: patients being treated with the drug fulvestrant (Faslodex(R)) have demonstrated significant interference in immunoassay methods for estradiol measurement. The cross reactivity could lead to falsely elevated estradiol test results leading to an inappropriate clinical assessment of estrogen status. Quest Diagnostics order code 30289-Estradiol, Ultrasensitive LC/MS/MS demonstrates negligible cross reactivity with fulvestrant.

FSH AND LH

Analyte	Value
FSH	5.0 Reference Range: 1.4-12.8 mIU/mL
LH	2.7 Reference Range: 1.5-9.3 mIU/mL

HEMOGLOBIN A1c

Analyte	Value
HEMOGLOBIN A1c	5.3 Reference Range: <5.7 %

For the purpose of screening for the presence of diabetes:

<5.7% Consistent with the absence of diabetes
5.7-6.4% Consistent with increased risk for diabetes (prediabetes)
> or =6.5% Consistent with diabetes

This assay result is consistent with a decreased risk of diabetes.

Currently, no consensus exists regarding use of hemoglobin A1c for diagnosis of diabetes in children.

According to American Diabetes Association (ADA) guidelines, hemoglobin A1c <7.0% represents optimal control in non-pregnant diabetic patients. Different metrics may apply to specific patient populations. Standards of Medical Care in Diabetes(ADA).

APOLIPOPROTEIN B

Analyte	Value
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APOLIPOPROTEIN B

56 mg/dL

Reference Range <90

Risk Category:

Optimal <90
Moderate 90-129
High > or = 130

A desirable treatment target may be <80 mg/dL or lower depending on the risk category of the patient including patients on lipid lowering therapies, patients with ASCVD, diabetes with >1 risk factors, Stage 3 or greater CKD with albuminuria, or heterozygous familial hypercholesterolemia. ApoB relative risk category cut points are based on AACE/ACE and ACC/AHA recommendations (Grundy SM, et al. 2019. doi:10.1016/j.jacc.2018.11.002; Handelsman Y, et al. 2020. doi:10.4158/CS-2020-0490).

Performing Sites

CB Quest Diagnostics-Wood Dale, 1355 Mittel Blvd, Wood Dale, IL 60191-1024 Laboratory Director: Kristie L White

Z3E MedFusion-MedFusion, 2501 South State Highway 121, Suite 1100, Lewisville, TX 75067-8065 Laboratory Director: Ithiel James L Frame MD, PhD

Key

 Priority Out of Range  Out of Range

These results have been sent to the person who ordered the tests. Your receipt of these results should not be viewed as medical advice and is not meant to replace discussion with your doctor or other healthcare professional.

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